DIGITALAIKART · INDIA EDITION

THE INDIAN THYROID PLAYBOOK

Hypothyroidism, subclinical thyroid, pregnancy
and the levothyroxine decisions that matter

42 million Indians. One in ten adults. Roughly one in three undiagnosed.
This is the plain-English playbook for the report in your hand:
how to read it, when to treat, how to take the tablet so it works,
what to do in pregnancy, and what to stop paying for.

2026-27 EDITION

19 SECTIONS

8 WORKSHEETS

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Every figure verified from ICMR, PIB, ITS, ATA and IRDAI sources

Before you start

This is a patient-education playbook, not a prescription. It is written to help you understand your own reports, ask better questions and stop wasting money, and it is deliberately honest about where the evidence is thin. It cannot examine you, and it does not know your kidneys, your heart, your pregnancy or your other medicines. Every decision about starting, changing or stopping a medicine belongs to a qualified doctor who has seen your reports. If this book and your doctor disagree about your specific case, your doctor wins.

Three things worth knowing about how it is written. First, numbers are sourced and dated: Indian data where Indian data exists, and international guidelines where it does not, with the disagreement between them left visible. Second, where the evidence says a treatment does less than the internet claims, that is stated plainly, because a book that flatters your hopes is worthless. Third, nothing here is a substitute for a repeat test.

How to use it

- **If you have just received an abnormal report,** read Chapter 3 first, then Chapter 5. That is the shortest path from panic to a plan.
- **If you have been on levothyroxine for years and feel no better,** read Chapter 6, then Chapter 9. Most of the answer is in the routine, not the dose.
- **If you are pregnant, planning a pregnancy, or recently delivered,** go straight to Chapter 10. Do not wait.
- **If someone in the family has a nodule,** Chapter 13 explains what the ultrasound report is actually saying.
- **If cost is the worry,** Chapter 15 is about tests, generic medicine and insurance rules.

The worksheets at the end are the part people actually keep. Print them, or copy the columns into a notebook. A dated record of your own numbers, in one place, is the single most useful thing you can bring to an appointment.

> *One rule holds this whole book together: a symptom is not a diagnosis, a single report is not a diagnosis, and a lifelong medicine deserves more than one blood test taken during a fever.*

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Chapters 1 to 5 explain the landscape and the decision. Chapters 6 to 8 are the operating manual for the medicine. Chapters 9 to 15 handle the situations where the standard advice fails. Chapter 16 dismantles the myths, Chapter 17 is the printable part, and Chapter 18 is the 90-day plan.

1. The thyroid problem in India, in numbers

Start with the size of the thing, because it changes how you should read your own report. In February 2026, speaking at the Indian Thyroid Society conference in Chandigarh, the Union Minister of State for Science and Technology put the number of Indians living with a thyroid disorder at about 42 million, and said hypothyroidism affects nearly 11 per cent of adults. He called the undiagnosed cases a silent threat to the country's workforce. That is not a marketing line from a supplement company. It is a government press release.

The best-known Indian prevalence study tested 5,376 adults across eight cities and found hypothyroidism in 10.95 per cent of them. Women were hit far harder than men: 15.86 per cent against 5.02 per cent. Inland cities had more hypothyroidism than coastal ones, 11.73 per cent against 9.45 per cent. And here is the part that matters for you personally: 3.47 per cent of that sample were diagnosed for the first time during the study. Roughly one in three people walking around with hypothyroidism in that study did not know it.

The same study found anti-TPO antibodies, the marker of autoimmune thyroid disease, in 21.85 per cent of participants. Subclinical hypothyroidism, which is a raised TSH with a normal free T4, was present in 8.02 per cent. A separate study of older adults in Bengaluru found hypothyroidism in 17.69 per cent of people aged 45 and above, and most of those cases were subclinical.

Group	What the Indian data shows	Source
Adults, general	10.95 per cent hypothyroid; 3.47 per cent undetected until tested	Eight-city study, Indian J Endocrinol Metab
Women	15.86 per cent, about three times the male rate	Same study
Pregnant women	17 per cent pooled prevalence; 15 per cent subclinical, 3 per cent overt	IJMR meta-analysis of 60 studies, 2026
Schoolchildren 10-18	Goitre 10.3 per cent; subclinical hypothyroidism 13 per cent; anti-TPO positive 10.4 per cent	ICMR-SALT-T 1, Lancet Regional Health SE Asia, 2026
Adults 45 and above	17.69 per cent in an urban south Indian cohort	Tata Longitudinal Study of Ageing

Pregnancy deserves its own paragraph. A 2026 systematic review of 60 Indian studies pooled the prevalence of hypothyroidism in pregnancy at 17 per cent, with subclinical disease at 15 per cent. In those pregnancies, preterm birth ran at 18 per cent and low birth weight at 17 per cent among women with subclinical hypothyroidism. This is why the first trimester is treated as urgent in a way that a mild TSH rise in a 45-year-old man is not.

Two things have changed in India over the last forty years. First, iodine. Universal salt iodisation began in 1986, and it worked: NFHS-5 found iodised salt in 94 per cent of households, and the 2026 ICMR schoolchildren study found a median urinary iodine of 133 mcg per litre, which is adequate. Goitre is falling. But hypothyroidism did not disappear with iodine. It changed shape. Autoimmune thyroid disease, not iodine deficiency, is now the main cause in most of India, and anti-TPO positivity is common even in children.

Second, testing. A thyroid profile is now a Rs. 300-800 add-on to any health check-up package, and apps push full-body panels every quarter. More testing means more numbers, and more numbers mean more people being handed a levothyroxine strip for a TSH of 5.6 that would have resolved on its own. The 2026 position is uncomfortable for everyone selling something: India has both a large undiagnosed burden and a large over-treated population, at the same time.

> *Your job in the next 40 pages is to work out which of those two you are, and to stop paying for the wrong one. That is a more useful goal than 'fixing your thyroid'.*

2. What your thyroid actually does

Your thyroid is a small butterfly-shaped gland sitting in the front of your neck, just below the Adam's apple. It weighs about 20 to 30 grams. It makes two hormones: thyroxine, called T4, and triiodothyronine, called T3. T4 is mostly a storage form. T3 is the version your cells actually use, and most of your T3 is not made by the thyroid at all; it is made inside other tissues by removing one iodine atom from T4.

The gland does not decide how much to make. Your pituitary, a pea-sized gland at the base of the brain, measures the hormone level in your blood and sends out thyroid stimulating hormone, TSH, as an instruction. High TSH means the pituitary thinks hormone levels are low, so it is shouting. Low TSH means it thinks levels are high, so it has gone quiet. This is a thermostat, and it is why TSH, not T4, is the test that tells you the most in most situations.

When thyroid hormone is low, almost every system slows down. Your resting energy use falls, so you feel cold when others do not. Your heart beats a little slower. Your gut moves more slowly, so you get constipated. Your skin and hair turn dry and brittle. Your cholesterol clearance drops, so LDL rises. Your menstrual cycle becomes heavier and less regular. Your thinking feels wrapped in cotton wool. None of this is dramatic, which is exactly the problem: hypothyroidism usually arrives as a slow fog, not an event.

Why so many people get blamed for the wrong thing

Because the symptoms are the same symptoms as a dozen other common Indian problems. Fatigue is also iron deficiency, which is extremely common in Indian women. Hair fall is also low ferritin and low protein intake. Low mood is also depression, sleep debt, an abusive job, or perimenopause. Constipation is also a low-fibre diet and not enough water. Weight gain is also, mostly, calories.

So the first discipline in this playbook is simple. A symptom is not a diagnosis. A test is not a diagnosis either. A diagnosis needs a number that is clearly abnormal, a clinical picture that fits, and a decision by a doctor who has looked at both. What you can do, and what most people in India do not do, is arrive at that appointment with the right information in the right order.

What causes hypothyroidism

- **Hashimoto's thyroiditis.** Your immune system slowly damages the gland. This is the commonest cause in iodine-sufficient India, and it shows up as a raised anti-TPO antibody. It runs in families and clusters with other autoimmune conditions such as type 1 diabetes, coeliac disease and vitiligo.

- **Iodine deficiency or, less often, iodine excess.** Still relevant in pockets of India, and now also in people who take kelp or iodine supplements.
- **After treatment for hyperthyroidism.** Radioactive iodine or thyroid surgery usually leaves you needing levothyroxine for life. That is expected, not a complication.
- **Medicines.** Amiodarone for heart rhythm, lithium for bipolar disorder, interferon, and the newer cancer immunotherapy drugs can all cause thyroid dysfunction. If you are on any of these, your thyroid should be checked on a schedule, not when you feel unwell.
- **Central hypothyroidism.** Rare. The thyroid is fine but the pituitary is not sending the signal. Here TSH can look normal or low while T4 is low, which is why a doctor who only reads TSH can miss it.
- **Congenital hypothyroidism.** Present from birth. India screens for it under the child health programme, and treatment in the first weeks of life prevents lifelong intellectual disability.
- **Transient causes.** Severe illness, recovery from surgery, some viral thyroiditis, and the postpartum period can all produce abnormal thyroid tests that settle by themselves. This is why one abnormal report is not a life sentence.

Hyperthyroidism, where the gland makes too much hormone, is a different condition with different treatment. It is covered here only where it overlaps with your story, mainly as over-replacement with levothyroxine and as the transient phase of postpartum thyroiditis.

3. Reading your thyroid report without panic

Most Indian labs report a thyroid profile with three or five numbers. Here is what each one is for, and what it cannot tell you.

Test	What it measures	How to read it
TSH	The pituitary's instruction to the thyroid	The primary screening test. High means underactive, low means overactive or over-treated.
Free T4 (FT4)	The unbound, usable thyroxine in your blood	Low with a high TSH means overt hypothyroidism. Normal with a high TSH means subclinical.
Free T3 (FT3)	The active hormone	Useful in hyperthyroidism and in rare cases. Not needed routinely in hypothyroidism.
Anti-TPO	Antibodies against thyroid peroxidase	Confirms an autoimmune cause and predicts progression. Not a measure of severity.
Anti-Tg	Antibodies against thyroglobulin	Mainly used in thyroid cancer follow-up. Rarely useful in ordinary hypothyroidism.

Units confuse people unnecessarily. TSH is reported as mIU/L or mcIU/mL, and these are the same thing. Some labs print a reference range of 0.4 to 4.5, others 0.3 to 5.5, others 0.5 to 6.0. The range is built from the local population the lab tested, which means it is a description of that population, not a law of your body. A TSH of 5.2 is 'high' in one lab and 'normal' in another. Your doctor should be reading the number with the range and your age in view.

The four patterns that cover most reports

TSH	Free T4	What it usually means	Usual next step
High	Low	Overt hypothyroidism	Start levothyroxine, with a dose suited to your age and heart status
High	Normal	Subclinical hypothyroidism	Repeat in 8-12 weeks; then decide, do not decide today
Low	High or normal	Hyperthyroidism, or too much levothyroxine	Check whether you are over-replaced; if not, investigate the thyroid itself
Low or normal	Low	Possible central hypothyroidism or non-thyroidal illness	Look at the pituitary and at your overall health, not just the thyroid

Three reports, read line by line

Report A. A 34-year-old woman, tired for six months, hair falling, cycles heavier. TSH 6.8 mIU/L (range 0.4-4.5), free T4 1.1 ng/dL (normal), anti-TPO 380 IU/mL (positive). This is subclinical hypothyroidism with an autoimmune cause. She is symptomatic, antibody positive, and planning a pregnancy within the year. That combination is a reasonable reason to start treatment, and also a reason to check ferritin before assuming the thyroid explains all the tiredness.

Report B. A 52-year-old man, 6 kg heavier than last year, constipated, cold all the time. TSH 9.4 mIU/L, free T4 0.7 ng/dL (low). This is overt hypothyroidism. It is treated, with a starting dose chosen around his weight, age and heart status, and a repeat test at six to eight weeks. Nothing here needs a second opinion before starting.

Report C. A 41-year-old woman on 125 mcg for three years, palpitations, losing weight, sleeping badly. TSH 0.08 mIU/L (low), free T4 1.8 ng/dL (high-normal). This is not a thyroid disease flaring up. It is likely over-replacement, and the right move is a dose reduction and a recheck, not a new diagnosis. Notice that the same test means something completely different depending on whether you are taking a tablet.

Three reports, three different actions, and only one of them is 'start a lifelong medicine'. That is what reading a report properly looks like.

Five things that make a thyroid test lie

- **Biotin.** The hair-and-nail supplement sold in every pharmacy interferes with many immunoassays and can make TSH look falsely low or falsely high. Stop biotin two to three days before a thyroid blood test. This is in the official US prescribing information for levothyroxine, and most Indian patients are never told.
- **Acute illness.** TSH drifts during and after any significant illness or surgery. Do not fine-tune a dose in that window.
- **Time of day.** TSH is highest in the early morning and falls during the day. Keep your tests at a similar time and, ideally, at the same lab.
- **Pregnancy.** The normal range shifts downwards from the first trimester. A TSH of 3.8 that is unremarkable in a man is worth attention at 8 weeks of pregnancy.
- **Antibody interference.** Rare, but real. If your TSH and your free T4 tell completely contradictory stories, ask whether the assay could be affected.

> *The single most useful sentence in this book: do not start a lifelong medicine on one report. The Indian Thyroid Society's own consensus on subclinical hypothyroidism asks for a repeat free T4 and TSH in 8 to 12 weeks before treatment, unless there is a compelling reason to start immediately, such as pregnancy or infertility treatment.*

What is a compelling reason? Pregnancy or planning one, symptoms that are severe and clearly fit, a goitre that is growing, or a TSH above 10. Everything else can wait eight weeks, and often normalises by itself.

4. Symptoms: what is thyroid and what is not

The honest symptom list for hypothyroidism looks like this: tiredness that sleep does not fix, feeling cold when others are comfortable, constipation, dry skin, hair thinning, heavier or irregular periods, low mood, slower thinking and word-finding, muscle aches, a hoarse voice, puffiness around the eyes, and weight gain of a few kilos that arrived without a change in eating.

The honest second half of that list is this: every one of those symptoms is also caused by something else that is very common in India. Iron deficiency anaemia affects a large share of Indian women. Vitamin B12 deficiency is widespread in vegetarian households. Vitamin D deficiency is near-universal in office workers. Sleep apnoea is common in anyone with a collar size above 17 inches and a partner who complains about snoring. Depression is common and treatable. Perimenopause rewrites sleep, mood and cycles for years.

In 2018, a meta-analysis in JAMA pooled 21 randomised trials covering 2,192 adults with subclinical hypothyroidism. Thyroid hormone lowered TSH into the normal range but produced no measurable improvement in quality of life or thyroid-related symptoms. The BMJ's 2019 rapid recommendation went further and advised against treating subclinical hypothyroidism in most adults. You will not hear this from the clinic that sells you a monthly 'thyroid care' package, but it is the best evidence we have, and it should shape how much you expect from a dose change.

What treatment does reliably fix

- Overt hypothyroidism: energy, cold intolerance, constipation, dry skin, mood and mental sharpness improve over weeks to months once the dose is right.
- Cholesterol: LDL usually falls once thyroid hormone is normal, sometimes by a meaningful margin.
- Menstrual regularity and fertility: cycles often normalise when thyroid status is corrected.
- Pregnancy outcomes: correcting hypothyroidism before and during pregnancy reduces the risk of preterm birth and low birth weight.

What treatment does not fix, whatever you were promised

- **Weight.** The Indian Thyroid Society consensus is blunt about this: the early weight change from starting levothyroxine is mostly fluid, and it is unlikely to exceed 5 kg even in obese patients, especially if your TSH was only modestly raised. Levothyroxine is not a weight-loss drug, and taking extra to lose weight is dangerous.

- **Hair fall.** It does improve, but hair grows slowly. Expect three to six months, and check ferritin, protein intake and B12 in the meantime.
- **Long-standing fatigue with a normal TSH.** If the number is in range and you still feel terrible, the answer is almost never a bigger dose. Chapter 9 is about what to look at instead.

This is why a symptom diary beats a symptom memory. Two weeks of two-line entries will tell you more than trying to remember how you felt in March. There is a printable version in the worksheets section.

> *Rule of thumb: treat the report when it is clearly abnormal, treat the person when the picture is clear, and never treat a single number in isolation.*

5. Overt and subclinical: the decision that matters most

Overt hypothyroidism means a high TSH with a low free T4. It is not controversial. It is treated, usually for life, and the only questions are the starting dose and how fast to raise it.

Subclinical hypothyroidism means a high TSH with a normal free T4. This is where most Indian thyroid prescriptions are written, and where most of the disagreement lives. Roughly 8 per cent of Indian adults sit in this category, and most of them feel perfectly well.

What the guidelines actually say

TSH level	What the evidence supports	Practical reading
Above 10 mIU/L	Treat. Consistent across Indian and international guidance.	Start levothyroxine and monitor.
4.5 to 10 with symptoms, goitre, positive anti-TPO, or cardiovascular risk	Treatment may be considered; individual judgement.	A reasonable trial, with a clear review date and a plan to stop if nothing changes.
4.5 to 10, no symptoms, antibody negative, under 70	Repeat the test in 8-12 weeks first. Many normalise.	Watch and recheck rather than start today.
4.5 to 10, over 70	Higher TSH is tolerated; treatment is often not needed.	Individualise; a target of about 3-5 mIU/L is acceptable if treated.
Pregnant or planning pregnancy	Treat. Different thresholds apply.	See Chapter 10 before you do anything else.

The Indian Thyroid Society's 2022 consensus on subclinical hypothyroidism adds detail that most patients never hear. Confirm with a repeat free T4 and TSH in 8 to 12 weeks before starting. Rule out the things that falsely raise TSH: recovery from illness, untreated adrenal insufficiency, heterophilic antibodies in the assay, and obesity, where TSH often normalises with weight loss. Aim for a TSH in the lower half of normal, around 0.5 to 2.0 mIU/L, if you do treat. In older patients, a TSH of 3 to 5 mIU/L is a reasonable target.

Why over-treatment is a real harm, not a technicality

The same consensus notes that over-replacement occurs in about 20 per cent of levothyroxine-treated patients. Too much thyroid hormone means palpitations and atrial fibrillation, bone loss and higher fracture risk in postmenopausal women, anxiety, insomnia and unintended weight loss. It is a slow, quiet harm, which is why it needs a number check rather than a feeling check.

The BMJ panel put it more strongly than any Indian guideline does: for most adults with subclinical hypothyroidism, the burden of lifelong medication and the uncertainty of harm outweigh a benefit that randomised trials have not been able to show. Their exceptions were women trying to conceive, a TSH above 20, severe symptoms, and very young adults. You do not have to agree with every word of that. You do have to know it exists before you accept a lifetime prescription for a TSH of 5.8.

> *The most expensive mistake in Indian thyroid care is not a missed diagnosis. It is 100 mcg taken for fifteen years for a TSH of 5.8 that was measured once, during a viral fever, on a Sunday morning, while on a biotin supplement.*

6. Levothyroxine: twelve rules that decide whether it works

Levothyroxine is a fussy medicine. It is absorbed in the jejunum and ileum, it needs stomach acid to dissolve properly, and its absorption drops sharply when it meets food, calcium, iron or antacids. Under ideal fasting conditions you absorb about 70 to 80 per cent of the dose. Under bad conditions you can absorb far less, and then everyone blames the dose, the brand, or your body.

1. **Take it on an empty stomach.** The American Thyroid Association's guideline recommends a consistent 60 minutes before breakfast, or at bedtime at least three hours after the evening meal. If 60 minutes is impossible, 30 minutes is a reasonable compromise; do not take it with breakfast.
2. **Water only.** Not chai, not milk, not coffee, not juice. Coffee and fibre-rich food measurably reduce absorption.
3. **Be consistent rather than perfect.** The same routine every day matters more than the ideal routine on some days. If bedtime suits your shifts, use bedtime every day.
4. **Keep a four-hour gap from calcium, iron, multivitamins, antacids, sucralfate, sevelamer, cholestyramine and orlistat.** This is the single most commonly broken rule in India, because the calcium tablet and the thyroid tablet are usually swallowed together after dinner.
5. **Proton pump inhibitors and antacids reduce absorption.** If you take pantoprazole or omeprazole daily, mention it; your dose may need revisiting.
6. **The full replacement dose is about 1.6 mcg per kg per day.** For a 60 kg adult that is roughly 100 mcg, but starting dose depends on age and heart status, not just weight.
7. **Start low if you are over 65 or have heart disease.** Usually 25 to 50 mcg, raised gradually. A young, otherwise healthy adult with overt hypothyroidism can start closer to full replacement.
8. **Wait 6 to 8 weeks before rechecking TSH.** The pituitary takes that long to settle. Testing at two weeks tells you nothing and tempts everyone into a pointless dose change.

9. **Stay on one brand and one strength where you can.** Different manufacturers have different bioavailability. If your pharmacy switches you, recheck TSH in 6 to 8 weeks.
10. **Store it properly.** Below 25 degrees Celsius, dry, away from the bathroom shelf and the kitchen steam. Indian summer and monsoon are hard on this tablet. Keep it in the strip, in a closed box.
11. **Do not double up routinely after a missed dose.** Take it when you remember on the same day. If the day is gone, skip it and continue as usual, unless your doctor has told you otherwise.
12. **Never stop on your own.** Not when you feel better, not when you lose weight, not because a friend says so. Stopping is a decision made with a doctor and a repeat test.

A timing plan that survives an Indian morning

Your situation	Workable routine	Watch out for
Office or shift worker, early start	Tablet at 6 am on waking, breakfast at 7 am	Chai at the bedside defeats the purpose
Late sleeper, no breakfast	Tablet at bedtime, at least 3 hours after dinner	Keep it far from the calcium tablet
Takes calcium or iron daily	Thyroid in the morning, calcium at lunch or dinner	Four hours apart, every day
Night shifts	Take it at the same point in your sleep cycle daily, and note the time	Consistency beats clock time
Frequent travel	Carry a strip in hand luggage; keep the same routine	Do not leave strips in a hot car or a checked bag

One more thing that nobody says clearly enough: levothyroxine is a narrow-therapeutic-index drug, which means small differences in how you take it can move your TSH a lot. If your TSH swings between 1.2 and 7.8 across six months without any dose change, the first suspect is not your gland. It is your routine, your brand, your calcium tablet, or the lab.

7. Dose, titration and your first six months

Levothyroxine comes in steps: 12.5, 25, 37.5, 50, 62.5, 75, 88, 100, 112, 125 and 150 mcg. Most Indian prescriptions settle somewhere between 50 and 125 mcg for adults. The steps exist so the dose can be moved in small increments, usually by 12.5 to 25 mcg at a time, based on TSH.

What the first six months actually look like

When	What is happening	What to do
Week 0	You start, or your dose changes	Fix the timing routine before you fix anything else
Weeks 2-4	Energy and bowel habits often improve first	Do not change the dose on how you feel in week 3
Week 6-8	TSH has settled on the new dose	Blood test, then review the number with your doctor
Weeks 8-16	Cholesterol, weight and skin continue to shift	Expect small changes, not transformation

When	What is happening	What to do
Months 3-6	Hair and mood improve most slowly	Check ferritin, B12 and vitamin D if hair is the main complaint

A realistic expectation, stated plainly: if you had overt hypothyroidism, the first three months can feel like a fog lifting. If you had a TSH of 6.5 and mild symptoms, the honest chance is that you will feel roughly the same, because that is what the trials found. Knowing this in advance is not pessimism. It stops you from chasing dose after dose and ending up over-replaced.

When the dose needs to go up

- Pregnancy, immediately and by 20 to 30 per cent on confirmation.
- Weight gain of more than 5 to 10 per cent of body weight.
- Starting oestrogen, including combined oral contraceptive pills or hormone therapy, which raise thyroid-binding globulin and increase requirement.
- Starting a proton pump inhibitor, iron, or calcium regularly.
- Bariatric surgery or any condition causing malabsorption, such as untreated coeliac disease.
- After radioiodine or thyroid surgery, where remaining function can decline over months.

When the dose may need to go down

- A TSH that has fallen below the reference range with palpitations, tremor, anxiety or loose stools.
- Significant weight loss.
- Stopping oestrogen.
- Aged over 70 with a suppressed TSH and no symptoms: over-replacement in this group carries heart and bone risk.

If you are on a high dose and your TSH will not come down, the Indian Thyroid Society's 2025 consensus on T3 calls this refractory hypothyroidism, defined as a TSH still above about 5.5 mIU/L despite more than 1.9 mcg per kg per day. It affects roughly 15 to 20 per cent of people on levothyroxine. The first step in that situation is not a new drug. It is checking whether you are actually taking the tablet as prescribed, then looking at absorption: acid suppression, coeliac disease, H. pylori, previous bowel surgery, and brand switching.

> *Before any dose increase above 2 mcg per kg per day, someone should be asking a boring question: are you swallowing this tablet the way it needs to be swallowed?*

8. Monitoring for life: the calendar that saves money

Once your dose is stable, thyroid monitoring is cheap and simple. It becomes expensive and confusing only when it is done randomly, at different labs, at different times of day, with no record.

Situation	How often to test TSH	What else to check
Just started or dose changed	6-8 weeks after the change	Free T4 if pregnant or if the picture is unclear
Stable adult	Every 6-12 months	Lipids once a year; BP and weight at every visit
Pregnant	Every 4 weeks until mid-pregnancy, then at least once around 30 weeks	Free T4, and anti-TPO once
Planning pregnancy	Before conception, aiming for TSH below 2.5	Anti-TPO, ferritin, vitamin D
Over 65	Every 6-12 months, watching for over-replacement	Bone health, heart rhythm
After thyroid surgery or radioiodine	Every 6-8 weeks until stable, then 6-12 monthly	Calcium after surgery; cancer follow-up as advised
On amiodarone, lithium or immunotherapy	Every 6 months, or as your doctor sets	Free T4 as well as TSH

Testing without wasting money

- When you are stable, a TSH alone is usually enough. A full thyroid profile every three months adds cost, not information.
- Use the same lab. Reference ranges and assays differ, and a 0.6 difference between labs is meaningless.
- Test in the morning, before your tablet, at roughly the same time each visit.
- Keep every report in one folder, physical or on your phone, with the date and dose written on it. Half of all thyroid confusion in India is a records problem.
- Skip the online 'thyroid reversal' panels that include reverse T3, iodine loading, and hair mineral analysis. None of these change the treatment decision in ordinary hypothyroidism.

The five-minute annual review

Once a year, sit down with your reports and answer four questions. Is my TSH in the target range for my age and situation? Has my dose changed more than twice in the year, and if so why? Am I taking anything that interferes, calcium, iron, a PPI, a supplement with biotin? And is there a symptom I have been ignoring that is not thyroid at all? Those four questions catch most of what goes wrong.

9. 'My TSH is normal and I still feel terrible'

This is the most common email a thyroid patient writes, and the most common place where money gets wasted. The number is in range, the doctor says you are fine, and you are not fine. Before anyone increases your dose, work through the list below in order. In practice, one of these explains most cases.

What to check	Why it fits the picture	Test
Iron deficiency	Causes exactly the same fatigue, hair fall and breathlessness on stairs; very common in Indian women	Ferritin, haemoglobin, MCV
Vitamin B12	Common in vegetarian diets; causes fatigue, tingling and poor concentration	Serum B12, or MMA if borderline
Vitamin D	Near-universal in indoor workers; aches and low mood	25-hydroxy vitamin D
Sleep apnoea	Snoring, unrefreshing sleep, daytime fog; strongly linked to weight	Ask a doctor; sleep study if suspected
Depression or anxiety	Overlaps almost completely with 'thyroid symptoms'	A proper clinical conversation, not a form
Coeliac disease	Clusters with autoimmune thyroid disease; causes fatigue and poor absorption	Tissue transglutaminase antibody, if symptoms suggest
Diabetes or prediabetes	Fatigue, weight change, thirst	HbA1c, fasting glucose
Perimenopause	Sleep, mood, cycles, temperature all shift	Clinical assessment, FSH if needed
Medication effects	Beta blockers, statins, antihistamines, antidepressants all cause fatigue	Review the full list with your doctor

The T3 question, answered honestly

Combination therapy with T4 and T3 is the most marketed idea in thyroid care right now. The Indian Thyroid Society's December 2025 consensus addresses it directly, and the answer is more careful than either side of the internet argument.

- Roughly one-third of people on levothyroxine continue to report hypothyroid-type symptoms despite normal blood levels. That is real and should not be dismissed.
- A trial of combined T4 and T3 may be considered in people on long-term levothyroxine with persistent symptoms, after other causes have been ruled out.
- It is not recommended in pregnancy or in people planning pregnancy, and it is contraindicated in cardiac arrhythmia.
- It is not a weight-loss treatment. Liothyronine has been misused for that, and the consensus warns about it.
- Monitoring on combination therapy needs TSH, free T4, free T3 and the free T4 to free T3 ratio, which means more tests and more visits.

Read that as: a considered trial in a defined subgroup, run by a doctor who will stop it if it does not work. Not a supplement you order online, and not something to demand at your first follow-up.

What to refuse

- **Desiccated animal thyroid extract.** Potency varies between batches, and the T3 content makes it hard to monitor. Modern guidelines do not recommend it as first-line therapy.
- **Iodine supplements and kelp tablets.** India is iodine-sufficient. Extra iodine does not fix hypothyroidism and can worsen autoimmune thyroid disease.
- **'Thyroid detox' programmes, herbal drops and 'gland activation' therapy.** None of these have trial evidence, and some have caused liver injury.
- **Repeated full-body panels every quarter.** More numbers, not more clarity.

> *If your TSH is 1.8 and you feel awful, the next useful test is probably ferritin, not a higher dose of thyroxine.*

10. Pregnancy, fertility and the first 1,000 days

Thyroid care in pregnancy is the one area where being early matters more than being perfect. In the first trimester the baby depends entirely on your thyroxine, because its own thyroid does not start working until around 12 weeks. Untreated hypothyroidism is linked with miscarriage, preterm birth, low birth weight and impaired neurodevelopment.

The Indian numbers are not small. A 2026 meta-analysis of 60 Indian studies put hypothyroidism in pregnancy at 17 per cent, with subclinical disease at 15 per cent. Among women with subclinical hypothyroidism, 18 per cent had a preterm birth and 17 per cent had a low-birth-weight baby. That is the reason the threshold for treating in pregnancy is lower than in anyone else.

Before pregnancy

- If you already take levothyroxine, get TSH checked before you start trying, and aim for below 2.5 mIU/L.
- Ask for anti-TPO once. If it is positive, your risk of needing a dose increase in pregnancy is higher, and you will be monitored more closely.
- Check ferritin and vitamin D too. Fatigue in early pregnancy is often iron, not thyroid.
- Do not stop your tablet because you read something online. Levothyroxine is safe in pregnancy and breastfeeding.

The moment pregnancy is confirmed

The American Thyroid Association's patient guidance is unusually practical here: if you already have hypothyroidism, your dose may be increased by 20 to 30 per cent as soon as pregnancy is confirmed, without waiting for the first appointment. The easiest way to do this is two extra tablets a week. Then get TSH tested and let your doctor fine-tune.

Stage	Target	Testing
Pre-conception, known hypothyroidism	TSH below 2.5 mIU/L	Before you start trying
First trimester	TSH below 2.5 mIU/L on treatment; upper limit about 4.0 when using a population range	Every 4 weeks
Second trimester	TSH below 2.5 to 3.0 mIU/L on treatment	Every 4 weeks until mid-pregnancy
Third trimester	Keep within the trimester range	At least once around 30 weeks
Postpartum	Back to pre-pregnancy dose, then recheck	TSH at about 6 weeks after delivery

Two numbers cause most of the confusion. The old rule of 2.5 as the upper limit of normal in early pregnancy has been revised: the 2017 ATA guideline uses about 4.0 mIU/L as the upper limit for a typical early pregnancy when local ranges are unavailable, because studies in India and elsewhere showed only a modest downward shift. But for a woman already on treatment, the target remains below 2.5. Those are two different questions, and mixing them up produces either needless treatment or missed treatment.

Subclinical hypothyroidism in pregnancy

The Indian Thyroid Society's pregnancy recommendations are specific. Treat if TSH is above 10. Treat if anti-TPO is positive and TSH is between 4 and 10. Treatment may be considered if anti-TPO is negative with TSH between 4 and 10, or if anti-TPO is positive with TSH between 2.5 and 4. A starting dose of about 1.2 mcg per kg per day is suggested for newly diagnosed subclinical disease, with adjustments every four weeks.

After delivery

- Most women return to their pre-pregnancy dose immediately after delivery, with TSH checked at about six weeks.
- Postpartum thyroiditis affects a proportion of women in the year after birth: a transient overactive phase, then an underactive phase, often with fatigue and hair loss that everyone blames on the baby. If you feel unwell at three to six months postpartum, ask for a TSH.
- Breastfeeding is safe on levothyroxine. The dose does not need to change because you are nursing.
- Iodine requirement rises to 280 mcg a day while breastfeeding.

The newborn

Congenital hypothyroidism occurs in about 1 in 1,031 Indian newborns, higher than most global estimates. It is the most common preventable cause of intellectual disability, and treatment started in the first two weeks of life produces normal development in most children. India does not yet have a universal, fully funded newborn screening programme, though the child health programme under the National Health Mission lists congenital hypothyroidism screening, and many private hospitals offer it.

If you are expecting, ask one question at the hospital: will my baby's TSH be screened, and how will I get the result? If the answer is vague, ask again. A screen-positive baby needs a confirmatory venous

sample, and roughly 70 per cent of screen-positive Indian newborns in the published literature did get retested, which means 30 per cent did not.

> *In pregnancy, the correct sequence is: test early, treat early, adjust every four weeks, and do not stop after delivery without a plan.*

11. Iodine and the Indian kitchen

India solved iodine deficiency at a national scale and then half-solved it again at the kitchen shelf. Universal salt iodisation began in 1986. NFHS-5 found iodised salt in 94 per cent of households, and the 2026 ICMR study of schoolchildren found median urinary iodine at 133 mcg per litre, which is adequate. The remaining problem is not the country. It is the packet you buy and how you store it.

Group	Recommended daily allowance	Upper limit
Adult man or woman	140 mcg	1,100 mcg
Pregnancy	220 mcg	1,100 mcg
Breastfeeding	280 mcg	1,100 mcg
Children 1-9 years	90 mcg	200-400 mcg depending on age

These are the ICMR-NIN figures from the 2024 revision of Nutrient Requirements for Indians. Note that they are lower than the older 150 mcg figure still quoted in many articles. The practical implication is simple: one teaspoon of properly iodised salt a day, within the 5 g daily salt limit that ICMR-NIN recommends, gets most adults close to the requirement.

How to buy salt in India

- **Look for the word iodised on the pack.** Food safety regulations require iodised salt to contain 15 to 30 parts per million of iodine, and the sale of common salt for direct human consumption is not permitted unless it is iodised.
- **Pink Himalayan salt and most rock salts are not iodised.** They are fine as a finishing salt. If they are your only salt, your iodine is coming from milk, eggs, fish and other foods, and that may not be enough.
- **Store it closed, dry and away from the stove.** Iodine in salt is volatile; moisture, heat and light all reduce what reaches your plate. A closed container in a dark cupboard beats an open bowl next to the burner.
- **Do not add salt at the very start of a long boil** if you can avoid it; add it closer to the end. Some iodine is lost to heat and steam over long cooking.
- **Do not chase iodine with supplements.** Above 1,100 mcg a day you are in the range where iodine can trigger or worsen thyroid dysfunction, particularly in people with autoimmune disease.

The supplement trap

Kelp tablets, iodine drops and 'thyroid support' capsules typically contain hundreds of micrograms of iodine, sometimes more than the daily upper limit in a single capsule. In a country that is iodine-sufficient, this is at best useless and at worst harmful. The same applies to selenium: small trials suggest possible benefit in autoimmune thyroiditis, but the evidence is not strong enough to recommend routine high-dose supplementation, and selenium excess has its own toxicity.

Goitrogens, and why your mother was half right

Cabbage, cauliflower, broccoli, radish and soy contain compounds that can interfere with thyroid hormone production in theory. In practice, cooking destroys much of the effect, and normal portions do not cause hypothyroidism in people with adequate iodine. Do not remove vegetables from your diet to protect your thyroid. That trade makes you worse, not better.

> *The Indian kitchen does not need a thyroid diet. It needs iodised salt, stored properly, in sensible amounts, with enough protein and iron alongside it.*

12. Food, weight and energy: what helps and what is sold to you

There is no thyroid diet. There is a normal, adequate Indian diet that supports the medicine you are taking, and there is a large industry selling the idea that food can replace it. This chapter separates the two.

The five things that actually matter

- **Protein at every meal.** Dal, curd, paneer, eggs, fish, chicken, soya. Hair, muscle and thyroid hormone transport all need it. Most Indian vegetarian plates are cereal-heavy and protein-light.
- **Iron and B12.** Iron deficiency mimics hypothyroidism so closely that many people are treated for the wrong one. Get ferritin checked before you blame your thyroid for fatigue.
- **Calcium, taken at the right time.** If you take calcium, take it at lunch or dinner, never with your thyroid tablet. Four hours apart.
- **Fibre and water for constipation.** The thyroid tablet helps over months; psyllium, fruit, vegetables, dal and two litres of water help this week.
- **Vitamin D.** Sunlight or a supplement as advised. Deficiency is near-universal in urban India and adds its own fatigue and aches.

Weight, told straight

Hypothyroidism lowers your resting energy expenditure, so you burn somewhat less at rest. Correcting it restores that, and the early change on the scale is mostly fluid. The Indian Thyroid Society is explicit that weight loss from treatment is unlikely to exceed 5 kg, even in obese patients, and that it is smaller when your TSH was only mildly raised.

That means the levers for weight are the ordinary ones, and they still work: a modest calorie deficit, resistance training twice a week to protect muscle, 7,000 to 10,000 steps, and sleep. What does not

work is taking extra thyroxine. It causes atrial fibrillation, bone loss and muscle wasting, and the weight comes back the moment you stop.

A week of Indian meals that supports the treatment

There is nothing exotic here, and nothing that claims to heal a gland. The point is protein at every meal, iron-rich foods with vitamin C, enough fibre, and the calcium tablet kept away from your morning dose. Rotate what you already cook.

Meal	Monday to Wednesday	Thursday to Sunday
Breakfast	Two eggs or a bowl of moong dal chilla, plus one fruit	Poha with peanuts and peas, or besan chilla with curd
Lunch	Dal, roti or rice, a green vegetable, curd	Rajma or chana, roti, salad, buttermilk
Evening	Roasted chana or a handful of nuts	Sprouts chaat or a small bowl of curd
Dinner	Paneer or fish with vegetables and one roti	Dal, seasonal vegetable, one roti, salad
Daily	Iodised salt in cooking; two litres of water	Calcium or iron tablet at lunch, never with the thyroid tablet

If you eat non-vegetarian food, two or three servings of fish or chicken a week make the iron and B12 side of this much easier. If you are vegetarian, add curd or paneer daily and get B12 checked rather than assumed. None of this replaces the tablet, and the tablet does not replace any of this.

Hair fall, in order of likelihood

1. Low ferritin, which is common and treatable. Check it before anything else.
2. Low protein intake, especially in women eating one cereal-heavy meal a day.
3. Thyroid status that is not yet corrected, or has been corrected only recently. Hair cycles take three to six months.
4. Postpartum hair loss, which is normal and temporary.
5. Telogen effluvium after illness, surgery, crash dieting or severe stress.
6. Androgenetic hair loss, which is genetic and needs a different treatment entirely.

What to skip

- Thyroid detox teas, glandular concentrates and 'activation' massages.
- Very low calorie diets that promise to reset your metabolism. They cost you muscle and make the fatigue worse.
- Eliminating gluten, dairy or nightshades without a diagnosis. If coeliac disease is a genuine concern, test for it properly rather than removing foods indefinitely.
- Ashwagandha and similar herbs marketed for thyroid support. The trial evidence is thin, and product quality in the Indian market is inconsistent.

> *Sleep is not a lifestyle accessory in thyroid care. Six hours a night for months will produce the same fatigue, weight gain and low mood that people blame on their gland.*

13. Nodules, goitre and the cancer question

A thyroid nodule is a lump in the gland. On ultrasound, a large share of adults have at least one, and the overwhelming majority are benign and never cause a problem. The word 'nodule' on a report is not a diagnosis of cancer, and it is not a reason to panic before anyone has even measured it.

The order of operations

1. TSH first. If it is low, the nodule may be overactive and the next test is a scan, not a biopsy.
2. Ultrasound of the thyroid and the neck lymph nodes, by someone who does this often.
3. The ultrasound report should describe size in three dimensions, composition, echogenicity, margins, calcification and shape, and comment on lymph nodes.
4. Fine needle aspiration, guided by ultrasound, only if the size and the sonographic pattern together justify it.

Ultrasound pattern	Estimated cancer risk	When a biopsy is advised
High suspicion: solid, hypoechoic, irregular margins, microcalcifications, taller than wide	70-90 per cent	At 1 cm or larger
Intermediate suspicion: solid, hypoechoic, smooth margins, no other suspicious features	10-20 per cent	At 1 cm or larger
Low suspicion: isoechoic or hyperechoic solid, or partly cystic with eccentric solid areas	5-10 per cent	At 1.5 cm or larger
Very low suspicion: spongiform or partly cystic without suspicious features	Under 3 per cent	Consider at 2 cm; watching is also reasonable
Benign: purely cystic	Under 1 per cent	No biopsy

These thresholds come from the American Thyroid Association's 2015 nodule guideline, which remains the reference standard used in Indian practice. The practical consequence is that a 6 mm nodule with benign features does not need a needle, and a 1.2 cm nodule with suspicious features does, regardless of how anxious either of you feels.

Goitre

A goitre is an enlarged thyroid. In India it used to mean iodine deficiency; today it more often means autoimmune disease or multiple nodules. A goitre that is stable, painless and not pressing on anything is usually watched. One that is growing quickly, or causing difficulty swallowing or breathing, or changing your voice, needs prompt assessment.

The red flags worth acting on within days

- A lump that has grown noticeably in a few weeks.
- Hoarseness or a voice change that persists.
- Difficulty swallowing, or a feeling of pressure when lying down.
- A hard, fixed lump that does not move when you swallow.
- Swollen lymph nodes in the neck alongside the lump.

- Any of these in a child or teenager, where the threshold for investigation is lower.

If it is cancer

Thyroid cancer is uncommon relative to how often nodules are found, and the commonest types have excellent long-term outcomes when treated properly. Treatment is usually surgery, sometimes followed by radioactive iodine, and then lifelong levothyroxine with TSH kept in a target range set by your treating team. Follow-up involves ultrasound and thyroglobulin testing at intervals. What you should not do is delay a biopsy because you are afraid of the answer, or accept surgery without a cytology report.

> *The correct response to 'nodule seen' is a good ultrasound report, not a surgery date and not a year of ignoring it.*

14. Special situations

Over 65

Thyroid requirements fall with age, and the risks of over-replacement rise: atrial fibrillation and bone loss matter more at 70 than at 30. Start at 25 to 50 mcg, raise slowly, and accept a TSH between 3 and 5 mIU/L if that is where you are comfortable and symptom-free. A suppressed TSH in an older person with no symptoms is a reason to reduce the dose, not to celebrate.

Children and teenagers

In children, hypothyroidism shows up as poor growth, falling school performance, constipation, dry skin, delayed puberty or early puberty. Treatment is weight-based and needs to be precise, because the growing brain and skeleton are sensitive to both under- and over-treatment. Adherence is the main practical problem in adolescents: a tablet that must be taken 60 minutes before breakfast in a school-going household needs a plan, not a reminder.

Diabetes and PCOS

Thyroid disease and diabetes travel together. Untreated hypothyroidism makes glucose control harder and raises LDL, and it can mask or mimic the fatigue of poorly controlled diabetes. If you have type 2 diabetes, an annual TSH is reasonable. In PCOS, thyroid dysfunction should be excluded before attributing every symptom to the ovaries, and TSH should be optimised before fertility treatment.

After thyroid surgery or radioiodine

Expect lifelong levothyroxine and expect the dose to be adjusted more than once in the first year. After surgery, calcium should be checked, because the parathyroid glands can be disturbed. After radioiodine for cancer, TSH targets are deliberately different from ordinary hypothyroidism and are set by your treating team.

Before thyroid surgery: ten questions

1. What does the cytology report say, in Bethesda terms, and what does that category mean for me?
2. Is the whole gland coming out, or one lobe? What will that mean for my voice and my calcium?

3. Who will do the operation, and how many of these do they do in a month?
4. Will the nerves to my voice box be monitored during surgery?
5. What is the plan if the final pathology differs from the biopsy?
6. Will I need radioactive iodine afterwards, and who decides?
7. What dose of levothyroxine will I start on, and when is my first TSH check?
8. What are the signs of low calcium I should watch for in the first two weeks?
9. What should I do if my voice is hoarse after a week?
10. What does follow-up look like for the next five years, and how much will it cost?

Medicines that change your thyroid

Medicine	What it does	What to do
Amiodarone	Can cause hypo- or hyperthyroidism; contains iodine	TSH and free T4 every 6 months while on it
Lithium	Can cause goitre and hypothyroidism	Annual TSH, more often if symptomatic
Interferon and immunotherapy	Can trigger thyroid dysfunction	Baseline and periodic testing as advised
Oestrogen, including COCs	Raises thyroid-binding globulin; may need a higher dose	Recheck TSH 6-8 weeks after starting
Proton pump inhibitors	Reduce levothyroxine absorption	Mention it; recheck TSH after starting
Iron and calcium	Bind levothyroxine in the gut	Four hours apart, always

Shift work

Night shifts break the '60 minutes before breakfast' rule for millions of Indian workers. The answer is not to skip the tablet. Anchor it to a fixed point in your sleep cycle, take it with water, and keep the gap before your first meal consistent. Tell your doctor your schedule, so that a slightly different TSH reading is interpreted with that context.

15. Money: tests, medicines and insurance

Thyroid care in India is, or should be, one of the cheapest long-term conditions you can have. It becomes expensive when it is done badly: repeated full panels, brand-hopping, unnecessary supplements, and emergency-room bills for a heart rhythm problem caused by over-replacement.

The medicine

Levothyroxine is a generic, off-patent molecule and one of the least expensive chronic medicines in the country. Jan Aushadhi Kendras stock thyroxine sodium; published Jan Aushadhi price lists put 50 mcg at roughly Rs. 45 to 65 for a pack of 100 tablets, against roughly Rs. 180 to 200 for branded packs of the same strength. At one tablet a day, that is a few rupees a month. Prices change, so check the pack in front of you rather than trusting any figure in a book, including this one.

Two cautions on saving money here. First, if you switch between generic and branded products, recheck TSH after six to eight weeks, because bioavailability differs. Second, do not switch to a product that is stored badly or sold loose without a batch number.

The tests

- A TSH alone is the cheapest and usually sufficient test when you are stable.
- A free T4 is worth adding in pregnancy, in suspected central hypothyroidism, and when TSH and symptoms disagree.
- Anti-TPO is worth doing once, to establish whether the cause is autoimmune, and once before pregnancy.
- Ask your lab for the rate card. The same panel can vary by a factor of three between a hospital, a chain lab and a standalone centre in the same city.
- Keep a single folder with every report, dated, with the dose written on it. It prevents repeat testing more reliably than any app.

Insurance

Health insurance and thyroid disease interact in three ways worth knowing. Under the IRDAI (Insurance Products) Regulations 2024, which came into force on 1 April 2024, the maximum waiting period for a disclosed pre-existing disease is 36 months, down from 48. The moratorium period, after which a policy or claim cannot be contested for non-disclosure or misrepresentation except in proven fraud, is now 60 months of continuous coverage, down from 96, and it counts time spent with previous insurers if you have ported.

- Disclose the diagnosis and the medicine when you buy a policy. Non-disclosure is the most common reason thyroid-related claims get questioned before the moratorium ends.
- Check the specific waiting period clause in your Customer Information Sheet, which every policy must now include.
- Thyroid surgery and radioiodine are hospitalisations and are usually covered after waiting periods. Outpatient tests and consultations generally are not, unless you have an OPD add-on.
- A fine needle aspiration is usually a day-care procedure; check whether day-care is covered.

Where public systems help

- Children: the child health screening programme under the National Health Mission includes screening for goitre and, as an optional condition, congenital hypothyroidism, with free treatment through district early intervention centres.
- Eligible families: hospitalisation under Ayushman Bharat can cover thyroid surgery for those with a valid card, subject to the scheme's package rules.
- Medicines: Jan Aushadhi Kendras for low-cost thyroxine, and generic prescribing at government facilities.

What to stop paying for

- Quarterly full thyroid profiles when your dose has not changed.
- Iodine supplements in an iodine-sufficient country.
- Imported desiccated thyroid.
- Online 'thyroid reversal' programmes that promise to get you off the medicine. Some people with subclinical disease can come off it under supervision; nobody should be sold a course that promises it.

> *The cheapest thing you can do for your thyroid is take the tablet correctly, at the same time every day, and test once a year instead of four times.*

16. Ten myths that cost Indian families money

These are the ten statements I would most like to delete from Indian WhatsApp groups. Each one is followed by what the evidence actually supports, and why the difference matters in rupees and in health.

Myth 1: 'Thyroid makes you fat'

Untreated hypothyroidism lowers resting energy expenditure and adds a few kilos, much of it fluid. Correcting it does not melt fat away. The Indian Thyroid Society's own consensus says weight loss from treatment is unlikely to exceed 5 kg even in obese patients, and is smaller still when the TSH was only mildly raised. If you are 20 kg above your healthy weight, the thyroid is not the main story, and no dose of thyroxine will be.

Myth 2: 'You must avoid cabbage, cauliflower and soy'

These foods contain goitrogens, which can interfere with thyroid hormone production in laboratory conditions and in people with severe iodine deficiency eating very large amounts. Cooking reduces the effect substantially, and normal portions in an iodine-sufficient household do not cause hypothyroidism. Removing vegetables from your plate costs you fibre, vitamins and money, and gains you nothing.

Myth 3: 'Pink Himalayan salt is better for your thyroid'

It is not iodised, and India's food safety rules require salt sold for direct human consumption to be iodised, at 15 to 30 parts per million. Rock and pink salts sit outside that protection. Use them for finishing if you like the taste, but keep an iodised salt as your cooking salt.

Myth 4: 'Once you start, you can never stop'

For overt hypothyroidism, usually true, because the gland is not coming back. For subclinical hypothyroidism, not necessarily. Some people started on a low dose for a borderline TSH can be reassessed after a year, and a supervised trial off treatment is a legitimate clinical decision. What is not legitimate is stopping on your own because you feel fine, or paying someone who promises to get you off the medicine in 90 days.

Myth 5: 'My TSH is normal, so nothing is wrong'

A normal TSH rules out primary hypothyroidism reasonably well. It does not rule out iron deficiency, B12 deficiency, sleep apnoea, depression, diabetes or perimenopause, all of which produce the same fatigue. If you feel unwell with a normal TSH, the answer is a wider look, not a bigger dose.

Myth 6: 'T3 is the better thyroid medicine'

Levothyroxine, which is T4, is first-line for a reason: it is stable, it converts to T3 where your body needs it, and it is easy to monitor. The Indian Thyroid Society's 2025 consensus allows a considered trial of combined T4 and T3 in people on long-term treatment with persistent symptoms after other causes are excluded. That is a narrow, doctor-supervised trial, not a superior product, and it is contraindicated in pregnancy and in cardiac arrhythmia.

Myth 7: 'Iodine drops will fix my thyroid'

India is iodine-sufficient. Median urinary iodine in the 2026 ICMR schoolchildren study was 133 mcg per litre. Extra iodine does not treat hypothyroidism and can worsen autoimmune thyroid disease. The tolerable upper limit for adults is 1,100 mcg a day, and some kelp products exceed it in a single capsule.

Myth 8: 'Thyroid tablets damage your bones and liver'

At a correct replacement dose, levothyroxine is simply replacing a hormone you are short of, and it is safe over decades. At an excessive dose it causes bone loss, atrial fibrillation and a raised risk of fracture in postmenopausal women. The harm comes from over-replacement, not from the medicine, which is why the annual TSH matters more than the annual supplement.

Myth 9: 'You have to stop the tablet during pregnancy'

The opposite is true. Pregnancy increases thyroid hormone requirement, and the dose usually rises by 20 to 30 per cent as soon as pregnancy is confirmed. Levothyroxine is safe in pregnancy and while breastfeeding. Stopping it is one of the few genuinely dangerous things a thyroid patient can do.

Myth 10: 'Test every month to be safe'

TSH takes six to eight weeks to settle after a dose change. Testing more often produces noise, unnecessary dose changes, and bills. Test six to eight weeks after a change, then every six to twelve months when stable, and every four weeks in pregnancy. That is the whole schedule.

> *Every myth on this list has the same shape: it promises control over something that is not under your control, and charges you for the feeling.*

17. Eight printable worksheets

Print these, or copy the layout into a notebook. They are designed to be filled in by hand in under five minutes each, because a tracker you do not use is worse than no tracker.

Worksheet 1: My numbers

Date	TSH	Free T4	Dose	Lab	How I felt (one line)

Worksheet 2: Dose change tracker

Date	Old dose	New dose	Why it changed	Next test due

Worksheet 3: The eight-week titration sheet

Use this each time the dose changes. Fill the review date on day one, so the test does not slip.

Week	Dose taken	Symptom score (1-10)	Notes
Start date: _____			
Week 2			
Week 4			

Week	Dose taken	Symptom score (1-10)	Notes
Week 6			
Week 8: TSH test due _____			

Worksheet 4: Daily timing planner

Item	My fixed time	Notes
Levothyroxine		Water only, empty stomach or bedtime
First meal or chai		At least 30-60 minutes later
Calcium or iron tablet		At least 4 hours away
Pantoprazole or antacid		Tell your doctor you take it
Other daily medicines		Check for interactions once

Worksheet 5: Two-week symptom and energy diary

Score energy 1 to 10 each evening, and write one line about sleep, food and anything unusual. Two weeks is enough to see a pattern.

Day	Energy (1-10)	Sleep hours	One line about today
1			
2			
3			
4			
5			
6			
7			
8			
9			
10			
11			
12			
13			
14			

Worksheet 6: Pregnancy and postpartum checklist

- Pre-conception TSH done, and below 2.5 if already on treatment.

- Anti-TPO done once.
- Ferritin and vitamin D checked.
- Dose increase plan agreed with the doctor before conception.
- TSH booked every four weeks from confirmation.
- Iodine intake reviewed: iodised salt in the kitchen, no iodine supplements.
- Postpartum: pre-pregnancy dose restored, TSH at six weeks.
- Newborn screening for congenital hypothyroidism confirmed with the hospital.

Worksheet 7: Doctor visit question sheet

1. My TSH is ___ and my free T4 is ___. What is the target for someone my age and situation?
2. Is the cause autoimmune? Do I need anti-TPO, and does the answer change anything?
3. If this is subclinical, can we repeat the test in 8 to 12 weeks before starting treatment?
4. What is my starting dose, and why that number?
5. What should I do if I miss a dose?
6. Which of my other medicines or supplements interfere, and how do I space them?
7. When exactly is my next TSH test due?
8. What symptoms would mean I should come back sooner rather than wait?

Worksheet 8: Red flags: go now, do not wait for the review

- Chest pain, palpitations with dizziness, or fainting.
- Severe breathlessness, or swelling of the face and legs with confusion or drowsiness, especially in an older person in winter. This is a medical emergency.
- A rapidly growing neck lump, voice change, or difficulty swallowing or breathing.
- Pregnancy with a TSH above the target range, or any new thyroid symptom in the first trimester.
- A newborn with prolonged jaundice, poor feeding, constipation or unusual sleepiness, where congenital hypothyroidism has not been excluded.

18. Your first 90 days

Most of what goes wrong in Indian thyroid care goes wrong in the first three months: a test taken at the wrong time, a dose started without a repeat, a tablet swallowed with calcium, and no review booked. This is the sequence that avoids all four. It assumes you have a report in hand and a doctor you can reach.

Week 0: get your paperwork into one place

- Put every thyroid report you have into one folder, physical or on your phone. Write the date and the dose you were on, on each one.
- Write down every medicine, supplement and vitamin you take, including the calcium, the multivitamin and the protein powder. This list matters more than most people think.
- Stop biotin for two to three days before any blood test.

- Book the tests your situation needs: TSH and free T4 for everyone; anti-TPO once; ferritin, B12 and vitamin D if fatigue or hair fall is the main complaint.

Week 1: the appointment

Take the folder, the medicine list and the question sheet from Chapter 17. Ask the four questions that decide everything: what is my diagnosis, is this overt or subclinical, do we repeat the test before starting, and what is my target TSH for my age and situation. Write the answers down in front of the doctor. If you leave without a review date, you do not have a plan.

Your situation	What should happen this week
Overt hypothyroidism	Start treatment. Fix the daily timing before you leave the clinic.
Subclinical, TSH above 10	Start treatment, with a review booked at 6-8 weeks.
Subclinical, TSH 4.5-10, symptomatic or antibody positive	A trial is reasonable. Agree the review date and the stopping rule in advance.
Subclinical, TSH 4.5-10, no symptoms	Repeat the test in 8-12 weeks before starting anything.
Pregnant or planning	Treat to a TSH below 2.5, and book testing every 4 weeks.

Weeks 2 to 5: build the habit, change nothing else

- Take the tablet the same way every single day. Water only. Nothing else within 30 to 60 minutes.
- Move calcium, iron and antacids at least four hours away, and tell whoever prescribes for you that you take levothyroxine.
- Fill in the two-week symptom diary. Do not adjust anything based on how you feel in week 3; TSH has not settled yet.
- Do not buy a supplement, a detox kit or an iodine drop this month. Nothing in that aisle will help you in the next four weeks.

Weeks 6 to 8: the first real test

Get TSH tested, at the same lab, in the morning, before your tablet. Then compare it with the target your doctor set. If it is in range, you stay on the dose and move to six-monthly testing. If it is not, the dose moves by 12.5 to 25 mcg and the eight-week clock restarts. That is the whole loop, and it usually takes two or three turns to land.

Weeks 9 to 12: sort out what the thyroid is not

By now the thyroid part is either stable or being titrated. This is the month to deal with the rest: ferritin, B12, vitamin D, sleep, weight, and the appointment you have been postponing for something that is not your thyroid at all. It is also the month to do the money review in Chapter 15 and put the annual test date in your calendar.

The compressed version, if you are pregnant

Skip the 90-day pacing. Test TSH and free T4 now, increase the dose by 20 to 30 per cent if you are already on treatment, start levothyroxine if you are newly diagnosed and above the pregnancy threshold, and repeat every four weeks until mid-pregnancy. There is no waiting eight weeks in pregnancy.

> *Ninety days, one folder, one timing routine, two blood tests and one review date. That is the entire intervention, and it is more than most people in India are offered.*

19. Glossary

The words your doctor uses, in plain English. Keep this page open during your next appointment.

Term	What it means
TSH	Thyroid stimulating hormone. Made by the pituitary, it tells the thyroid how much hormone to produce. The main screening test.
T4 / thyroxine	The main hormone the thyroid makes. Levothyroxine tablets are synthetic T4.
T3 / triiodothyronine	The active hormone. Most of it is made in your tissues by converting T4.
Free T4	The unbound fraction of T4 in blood, which is the part available to your cells.
Hypothyroidism	Too little thyroid hormone. Symptoms plus a high TSH.
Hyperthyroidism	Too much thyroid hormone made by the gland itself.
Subclinical hypothyroidism	A high TSH with a normal free T4.
Overt hypothyroidism	A high TSH with a low free T4.
Euthyroid	Thyroid function is normal. Not a diagnosis, just a state.
Anti-TPO	Antibodies against thyroid peroxidase, a marker of autoimmune thyroid disease such as Hashimoto's.
Hashimoto's thyroiditis	Autoimmune inflammation of the thyroid; the commonest cause of hypothyroidism in India today.
Goitre	An enlarged thyroid gland.
Nodule	A lump within the thyroid. Common, and usually benign.
FNA	Fine needle aspiration. A thin needle sample of a nodule, usually ultrasound-guided, to look at cells.
Titration	Adjusting the dose in small steps based on repeat tests.
Refractory hypothyroidism	A TSH that stays high despite a dose above about 1.9 mcg per kg per day. Usually a compliance or absorption problem.
Over-replacement	A dose that pushes TSH below the reference range. Causes palpitations, bone loss and anxiety.
Postpartum thyroiditis	Transient thyroid inflammation in the year after delivery, often an overactive phase followed by an underactive one.

Term	What it means
Congenital hypothyroidism	Hypothyroidism present from birth. Screening and early treatment prevent intellectual disability.
Non-thyroidal illness	Abnormal thyroid tests caused by another illness, which usually normalise when you recover.
Radioiodine (RAI)	Radioactive iodine treatment, used for hyperthyroidism and some thyroid cancers. Often leads to lifelong levothyroxine.
Moratorium period	Under IRDAI rules, 60 months of continuous health cover after which a policy or claim cannot be contested for non-disclosure, except in proven fraud.

Sources and further reading

Every figure in this playbook comes from the documents below. Where Indian and international guidance differ, both are stated so you can see the disagreement rather than a smoothed-over version of it.

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This edition is current to November 2026. Guidelines change, drug labels change, and your own numbers change. If something in this book conflicts with what your treating doctor says about your specific case, your doctor wins. That is not a disclaimer; it is how medicine is supposed to work.